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Psoríase

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O que é?

Psoríase é uma doença inflamatória da pele, crônica, não contagiosa, multigênica (vários genes envolvidos), com incidência genética em cerca de 30% dos casos. Caracteriza-se por lesões avermelhadas e descamativas, normalmente em placas, que aparecem, em geral, no couro cabeludo, cotovelos e joelhos.

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O que causa?

- Genética
- Fatores psicológicos - ansiedade
- Estresse
- Exposição ao frio
- Uso de certos medicamentos
- Álcool
- Alimentos inflamatórios - glúten ?
- Açúcar
- Fumo
- Infecções

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Review > Clin Rev Allergy Immunol. 2016 Jun;50(3):377-89. doi: 10.1007/s12016-016-8535-x.

The Inflammatory Response in Psoriasis: a Comprehensive Review

Yaxiong Deng [1], Christopher Chang [2], Qianjin Lu [3] [4]

Affiliations + expand

PMID: 27025861 DOI: 10.1007/s12016-016-8535-x

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Review > Int J Mol Sci. 2022 Apr 13;23(8):4457. doi: 10.3390/ijms23084457.

Psoriasis and Systemic Inflammatory Disorders

Tomoko Tashiro [1], Yu Sawada [1]

Affiliations + expand

PMID: 35457278 PMCID: PMC9028262 DOI: 10.3390/ijms23084457

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Review > Antioxidants (Basel). 2022 Jan 29;11(2):282. doi: 10.3390/antiox11020282.

The Involvement of Oxidative Stress in Psoriasis: A Systematic Review

Elena-Codruța Dobrică [1 2], Matei-Alexandru Cozma [3 4], Mihnea-Alexandru Găman [3 5], Vlad-Mihai Voiculescu [2 3], Amelia Maria Găman [6 7]

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Insulina

Observational Study > Arch Dermatol Res. 2022 Nov;314(9):869-880.

doi: 10.1007/s00403-021-02298-9. Epub 2021 Nov 24.

Association between baseline insulin resistance and psoriasis incidence: the Women's Health Initiative

Alfred A Chan # 1, Houmin Li # 2, Wendy Li 3, Kathy Pan 4, Jennifer K Yee 5 6, Rowan T Chlebowski 4, Delphine J Lee 7 8 9

Affiliations + expand

PMID: 34816303 PMCID: PMC9512862 DOI: 10.1007/s00403-021-02298-9

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Review > Nutrients. 2022 Jul 20;14(14):2970. doi: 10.3390/nu14142970.

Gut Microbiota in Psoriasis

Mihaela Cristina Buhaș [1], Laura Ioana Gavrilaș [2], Rareș Candrea [3], Adrian Cătinean [4], Andrei Mocan [5], Doina Miere [2], Alexandru Tătaru [1]

Affiliations + expand

PMID: 35889927 PMCID: PMC9321451 DOI: 10.3390/nu14142970

Free PMC article

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Abstract

Psoriasis is a chronic inflammatory skin disease with autoimmune pathogenic characteristics and is caused by chronic inflammation, which results in uncontrolled keratinocyte growth and defective differentiation. The link between the gut microbiota and immune system regulation opened a novel angle to understand the pathogenesis of many chronic multifactorial diseases, including psoriasis. Current evidence suggests that modulation of the gut microbiota, both through dietary approaches and through supplementation with probiotics and prebiotics, could represent a novel therapeutic approach. The present work aims to highlight the latest scientific evidence regarding the microbiome alterations of psoriatic patients, as well as state of the art insights in terms of microbiome-targeted therapies as promising preventive and therapeutic tools for psoriasis.

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> Postepy Dermatol Alergol. 2022 Apr;39(2):368-374. doi: 10.5114/ada.2021.107098.
Epub 2021 Jun 20.

Clinical implications of intermittent Ramadan fasting on stable plaque psoriasis: a prospective observational study

Nawaf Almutairi [1], Dalia Shaaban [2]

Affiliations + expand

PMID: 35645682 PMCID: PMC9131972 DOI: 10.5114/ada.2021.107098

Free PMC article

Abstract

Introduction: Psoriasis is a chronic immune-mediated inflammatory disease. Environmental factors including diet have been supposed to play a role in its pathogenesis. Fasting in Ramadan consists of intermittent fasting in which participating Muslims refrain from eating and drinking from dawn up to sunset.

Aim: To validate the possible clinical consequences of Ramadan fasting for patients with psoriasis.

Material and methods: The study was conducted in Ramadan 2019. It included patients over 18 years who were diagnosed with stable chronic plaque psoriasis. PASI and BSA scores, body mass index and biochemical tests (including blood lipids, fasting blood glucose) were compared before and after a month of Ramadan fasting.

Results: The study included 121 psoriasis patients. The mean PASI score was 4.36 ±3.22 at the beginning of the study, which was reduced to 3.51 ±1.26 at the end of the study. The difference was statistically significant (p = 0.001). There was no difference in the change of weight. We found a statistically significant difference in fasting plasma glucose, HDL and triglycerides. Only mild adverse events were noted during the study period.

Conclusions: Ramadan intermittent fasting has beneficial effects on severity of the disease in psoriasis patients with a reduction in PASI score and no serious health hazards. Thus, Ramadan intermittent fasting could be considered during treatment of psoriasis patients.

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Jejum

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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Review > Antioxidants (Basel). 2021 Jan 22;10(2):157. doi: 10.3390/antiox10020157.

The Effect of Antioxidant and Anti-Inflammatory Capacity of Diet on Psoriasis and Psoriatic Arthritis Phenotype: Nutrition as Therapeutic Tool?

Pelagia Katsimbri [1], Emmanouil Korakas [2], Aikaterini Kountouri [2], Ignatios Ikonomidis [3], Elias Tsougos [4], Dionysios Vlachos [5], Evangelia Papadavid [6], Athanasios Raptis [2], Vaia Lambadiari [2]

Affiliations + expand

PMID: 33499118 PMCID: PMC7912156 DOI: 10.3390/antiox10020157

Free PMC article

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Abstract

Chronic inflammation and increased oxidative stress are contributing factors to many non-communicable diseases. A growing body of evidence indicates that dietary nutrients can activate the immune system and may lead to the overproduction of pro-inflammatory cytokines. Fatty acids as macronutrients are key players for immunomodulation, with n-3 polyunsaturated fatty acids having the most beneficial effect, while polyphenols and carotenoids seem to be the most promising antioxidants. Psoriasis is a chronic, immune-mediated inflammatory disease with multifactorial etiology. Obesity is a major risk factor for psoriasis, which leads to worse clinical outcomes. Weight loss interventions and, generally, dietary regimens such as gluten-free and Mediterranean diet or supplement use may potentially improve psoriasis' natural course and response to therapy. However, data about more sophisticated nutritional patterns, such as ketogenic, very low-carb or specific macro- and micro-nutrient substitution, are scarce. This review aims to present the effect of strictly structured dietary nutrients, that are known to affect glucose/lipid metabolism and insulin responses, on chronic inflammation and immunity, and to discuss the utility of nutritional regimens as possible therapeutic tools for psoriasis and psoriatic arthritis.

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Review > Int J Mol Sci. 2020 Jul 29;21(15):5405. doi: 10.3390/ijms21155405.

Nutrition and Psoriasis

Naoko Kanda [1], Toshihiko Hoashi [2], Hidehisa Saeki [2]

Affiliations + expand

PMID: 32751360 PMCID: PMC7432353 DOI: 10.3390/ijms21155405

Free PMC article

Abstract

Psoriasis is a chronic inflammatory skin disease characterized by accelerated tumor necrosis factor-α/interleukin-23/interleukin-17 axis, hyperproliferation and abnormal differentiation of epidermal keratinocytes. Psoriasis patients are frequently associated with obesity, diabetes, dyslipidemia, cardiovascular diseases, or inflammatory bowel diseases. Psoriasis patients often show unbalanced dietary habits such as higher intake of fat and lower intake of fish or dietary fibers, compared to controls. Such dietary habits might be related to the incidence and severity of psoriasis. Nutrition influences the development and progress of psoriasis and its comorbidities. Saturated fatty acids, simple sugars, red meat, or alcohol exacerbate psoriasis via the activation of nucleotide-binding domain, leucine-rich repeats containing family, pyrin domain-containing-3 inflammasome, tumor necrosis factor-α/interleukin-23/interleukin-17 pathway, reactive oxygen species, prostanoids/leukotrienes, gut dysbiosis or suppression of regulatory T cells, while *n*-3 polyunsaturated fatty acids, vitamin D, vitamin B12, short chain fatty acids, selenium, genistein, dietary fibers or probiotics ameliorate psoriasis via the suppression of inflammatory pathways above or induction of regulatory T cells. Psoriasis patients are associated with dysbiosis of gut microbiota and the deficiency of vitamin D or selenium. We herein present the update information regarding the stimulatory or regulatory effects of nutrients or food on psoriasis and the possible alleviation of psoriasis by nutritional strategies.

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> Clin Dermatol. 2022 Oct 1;S0738-081X(22)00124-9. doi: 10.1016/j.clindermatol.2022.09.008.
Online ahead of print.

Veganism in acne, atopic dermatitis, and psoriasis: Benefits of a plant-based diet

Marta Fusano [1]

> JAMA Dermatol. 2018 Sep 1;154(9):1017-1024. doi: 10.1001/jamadermatol.2018.2127.

Association Between Mediterranean Anti-inflammatory Dietary Profile and Severity of Psoriasis: Results From the NutriNet-Santé Cohort

Céline Phan [1], Mathilde Touvier [2], Emmanuelle Kesse-Guyot [2], Moufidath Adjibade [2], Serge Hercberg [2 3], Pierre Wolkenstein [1 4], Olivier Chosidow [1 4 5], Khaled Ezzedine [1 4], Emilie Sbidian [1 4 5]

Affiliations + expand

PMID: 30046840 PMCID: PMC6143029 DOI: 10.1001/jamadermatol.2018.2127

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Review > Dig Dis. 2019;37(6):451-457. doi: 10.1159/000500116. Epub 2019 May 10.

Psoriasis and Inflammatory Bowel Disease

Mario Cottone [1], Chiara Sapienza [2], Fabio Salvatore Macaluso [2], Marco Cannizzaro [2]

Review > Int J Mol Sci. 2022 Aug 13;23(16):9063. doi: 10.3390/ijms23169063.

Psoriasis and Cardiometabolic Diseases: Shared Genetic and Molecular Pathways

Stefano Piaserico [1], Gloria Orlando [1], Francesco Messina [1]

Affiliations + expand

PMID: 36012327 PMCID: PMC9409274 DOI: 10.3390/ijms23169063

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Review > Front Immunol. 2018 Apr 5;9:579. doi: 10.3389/fimmu.2018.00579. eCollection 2018.

Systemic Inflammation and Cardiovascular Comorbidity in Psoriasis Patients: Causes and Consequences

Wolf-Henning Boehncke [1] [2]

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Review > Cureus. 2021 Jan 12;13(1):e12653. doi: 10.7759/cureus.12653.

Etiological Association Between Psoriasis and Thyroid Diseases

Srilatha Eapi [1], Rupak Chowdhury [2], Odunayo S Lawal [3], Nimisha Mathur [3], Bilal Haider Malik [1]

Affiliations + expand

PMID: 33585138 PMCID: PMC7872875 DOI: 10.7759/cureus.12653

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Meta-Analysis > J Am Acad Dermatol. 2020 Jun;82(6):1376-1386.

doi: 10.1016/j.jaad.2019.11.039. Epub 2019 Dec 4.

Association between psoriasis and celiac disease: A systematic review and meta-analysis

Prakash Acharya [1], Mahesh Mathur [2]

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Review > Clin Dermatol. 2018 Jan-Feb;36(1):21-28. doi: 10.1016/j.clindermatol.2017.09.005. Epub 2017 Sep 8.

Psoriasis and the metabolic syndrome

Paolo Gisondi [1], Anna Chiara Fostini [2], Irene Fossà [3], Giampiero Girolomoni [2], Giovanni Targher [3]

Affiliations + expand

PMID: 29241748 DOI: 10.1016/j.clindermatol.2017.09.005

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> JAMA Dermatol. 2022 Jul 1;158(7):749-753. doi: 10.1001/jamadermatol.2022.1609.

Association Between Psoriasis and Nonalcoholic Fatty Liver Disease Among Outpatient US Adults

Zhijie Ruan [1 2], Tao Lu [1], Yanxia Chen [1], Mengsi Yuan [1], Haoyang Yu [1], Ruimin Liu [1], Xiaoping Xie [1]

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Suplementos

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CSC

- Ômega 3 (EPA)
- Glúten
- Vitamina D3
- Se, vit. A, Zn, Fe, Cu, Mn, vit. C, vit. E
- Fibras
- Flavonóides (ext. semente uva, óleo de uva)
- Cominho negro
- Silimarina
- ECGC, L teanina
- Polypodium leucotomos

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Review > Int J Dermatol. 2021 Dec;60(12):e493-e499. doi: 10.1111/ijd.15615.
Epub 2021 Apr 26.

A review of Nigella sativa plant-based therapy in dermatology

Jacqueline R Hwang [1], Alexander M Cartron [1], Amor Khachemoune [2 3]

Affiliations + expand

PMID: 33899217 DOI: 10.1111/ijd.15615

Abstract

Nigella sativa (N. sativa) is a widely used medicinal herb with a rich cultural and religious history in Unani, Ayurveda, Chinese, and Arabic medicine. N. sativa contains many natural bioactive agents including alkaloids, saponins, alpha-hederin, and thymoquinone that contribute to its broad range of benefits as a diuretic, bronchodilator, antihypertensive, antidiabetic, and analgesic. In addition, N. sativa possesses antimicrobial, anti-inflammatory, and antineoplastic effects, making it an interesting potential therapy for the treatment of dermatological conditions. This article reviews the current literature surrounding the pharmacological effects of N. sativa for the treatment of acne vulgaris, melanoma, vitiligo, atopic dermatitis, plaque psoriasis, and wound healing.

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Now Foods, Óleo da Semente de Cominho Preto, 1.000 mg, 60 Cápsulas Gelatinosas

Source Naturals, Óleo de Cominho Preto, 120 Cápsulas Softgel

Life Extension, Óleo da Semente de Cominho Preto, 60 Cápsulas Softgel

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Review > J Ethnopharmacol. 2022 Oct 28;297:115522. doi: 10.1016/j.jep.2022.115522.
Epub 2022 Jul 22.

Psoriasis treatment using Indigo Naturalis: Progress and strategy

Qianhui Zhang [1], Jin Xie [1], Gefei Li [1], Fang Wang [2], Junzhi Lin [3], Ming Yang [2], Aiai Du [4], Dingkun Zhang [5], Li Han [6]

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Review > J Drugs Dermatol. 2016 Feb;15(2):224-8.

Polypodium Leucotomos--An Overview of Basic Investigative Findings

Brian Berman, Charles Ellis, Craig Elmets

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JID Innov. 2022 Dec 16;3(3):100175. doi: 10.1016/j.xjidi.2022.100175. eCollection 2023 May.

Targeting TGF-β1/miR-21 Pathway in Keratinocytes Reveals Protective Effects of Silymarin on Imiquimod-Induced Psoriasis Mouse Model

Elodie Henriet [1 2], Florence Abdallah [1 2], Yoan Laurent [1 2], Cyril Guimpied [1 2], Emily Clement [3], Michel Simon [3], Chantal Pichon [1 2], Patrick Baril [1 2]

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Molecules. 2019 Jan; 24(1): 123.
Published online 2018 Dec 30. doi: 10.3390/molecules24010123
PMCID: PMC6337225
PMID: 30598040

Modulation of Skin Inflammatory Response by Active Components of Silymarin

Jana Juráňová,[1,2] Juliette Aury-Landas,[3] Karim Boumediene,[3] Catherine Baugé,[3] David Biedermann,[4] Jitka Ulrichová,[1,2] and Jana Franková[1,2,*]

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Abstract

Go to:

In this study, we compared selected silymarin components, such as quercetin (QE), 2,3-dehydrosilybin (DHS) and silybin (SB), with the anti-inflammatory drug indomethacin (IND) in terms of their wound healing potential. In view of the fact that pathological cutaneous wound healing is associated with persistent inflammation, we studied their anti-inflammatory activity against inflammation induced by bacterial lipopolysaccharide (LPS). We investigated the regulation of crucial pro-inflammatory transcription factors—nuclear factor kappa-B (NF-κB) and activator protein 1 (AP-1)—as well as the expression of downstream inflammatory targets by Western blotting, real-time PCR (RT-PCR), electrophoretic mobility shift assay (EMSA), and/or enzyme-linked immunosorbent assay (ELISA) in vitro using primary normal human dermal fibroblasts (NHDF). We demonstrated the greater ability of DHS to modulate the pro-inflammatory cytokines production via the NF-κB and AP-1 signaling pathways when compared to other tested substances. The prolonged exposure of LPS-challenged human dermal fibroblasts to DHS had both beneficial and detrimental consequences. DHS diminished interleukin-6 (IL-6) and interleukin-8 (IL-8) secretion but induced the significant upregulation of IL-8 mRNA associated with NF-κB and AP-1 activation. The observed conflicting results may compromise the main expected benefit, which is the acceleration of the healing of the wound via a diminished inflammation.

Clin Cosmet Investig Dermatol. 2014; 7: 321–327.
Published online 2014 Nov 11. doi: 10.2147/CCID.S69240
PMCID: PMC4235203
PMID: 25419153

A cosmeceutical formulation based on boswellic acids for the treatment of erythematous eczema and psoriasis

Stefano Togni,[1] Giada Maramaldi,[1] Francesco Di Pierro,[2] and Massimo Biondi[3]

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Background

Boswellic acids (BAs) show anti-inflammatory properties in a variety of inflammatory diseases, including rheumatoid arthritis, osteoarthritis, and asthma. A topical administration route is currently used to deliver active compounds in psoriatic and eczematous patients. In this double-blind study we compare a novel BA formulation (containing Bosexil®, INCI [International Nomenclature of Cosmetic Ingredients]: lecithin, *Boswellia serrata* resin extract) with a placebo formulation. A third arm of the trial received a formulation of *Vaccinium myrtillus* seed oil, previously demonstrated as an effective local treatment for psoriatic lesions.

Methods

Patients with psoriasis or erythematous eczema were randomly assigned, in a 1:1:1 ratio, to Bosexil®, *V. myrtillus* seed oil, or placebo. In order to evaluate the effects of treatment, the changes of scales and erythema from diagnosis to the end of treatment were scored in psoriatic patients, while changes in itch and erythema were analyzed for erythematous eczema patients. Psoriasis Area Severity Index and Eczema Area and Severity Index scores were also calculated.

Results

In patients with psoriasis, scales and erythema improved both with Bosexil® and the *V. myrtillus* seed oil treatment in comparison with placebo. In particular, the treatment with Bosexil® formulation improved scales (70% of cases) and erythema (50% of cases) without any case of worsening. In patients with eczema, the administration of placebo did not result in any improvement in 90% of cases, and in the remaining 10% worsened both itch and erythema. Bosexil® formulation improved both itch (60% of cases) and erythema (60% of cases) without any case of worsening. *V. myrtillus* seed oil improved itch and erythema in 66.7% and 77.8% of patients, respectively.

Conclusion

A topical formulation of Bosexil® may be promising for the treatment of psoriasis and erythematous eczema. Long-term studies are recommended to evaluate the adherence to this topical treatment and its clinical benefits in real life.

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> J Dtsch Dermatol Ges. 2010 Oct;8(10):788-96. doi: 10.1111/j.1610-0387.2010.07496.x. Epub 2010 Aug 5.

Which plant for which skin disease? Part 1: Atopic dermatitis, psoriasis, acne, condyloma and herpes simplex

[Article in English, German]

Juliane Reuter [1], Ute Wölfle, Steffi Weckesser, Christoph Schempp

Affiliations + expand

PMID: 20707875 DOI: 10.1111/j.1610-0387.2010.07496.x

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Abstract

Plant extracts and isolated compounds are increasingly used in cosmetics and food supplements to improve skin conditions. We first introduce the positive plant monographs with dermatological relevance of the former German Commission E. Subsequently clinical studies with botanicals for atopic dermatitis, psoriasis, acne, condylomata acuminata and herpes simplex are discussed. The best studies have been conducted with atopic dermatitis and psoriasis patients. Mahonia aquifolium, Hypericum perforatum, Glycyrrhiza glabra and certain traditional Chinese therapies have been shown to be effective in the treatment of atopic dermatitis. Mahonia aquifolium, Indigo naturalis and Capsicum frutescens are effective treatments for psoriasis. Green tea extract and tea tree oil have been investigated in the treatment of acne. Podophyllin and green tea extract are effective treatments for condylomata acuminata. Balm mint and a combination of sage and rhubarb have been shown to be effective in the treatment of herpes simplex in proof of concept studies.

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Review > Pharm Biol. 2012 Dec;50(12):1573-91. doi: 10.3109/13880209.2012.690430.
Epub 2012 Sep 12.

Plants and plant products with potential antipsoriatic activity--a review

Arshdeep Kaur [1], Suresh Kumar

Affiliations + expand

PMID: 22971237 DOI: 10.3109/13880209.2012.690430

Abstract

Context: Psoriasis vulgaris is a hyper proliferative, autoimmune skin disorder affecting 1-3% of the world's population. The prescribed synthetic drugs for the treatment of psoriasis are associated with severe side effects, thus, researchers around the globe are searching for new, effective, and safer drugs from natural resources.

Objective: The present review has been prepared with an objective to compile exhaustive literature on pharmacological reports on antipsoriatic plants, plant products, and formulations. An attempt has been made to incorporate chemical constituents (with structures) isolated from different plants responsible for antipsoriatic activity and their possible mechanism of actions in this review.

Materials and methods: The review has been compiled using references from major databases like Chemical Abstracts, Medicinal and Aromatic Plants Abstracts, PubMed, Scirus, Google scholar, Open J Gate, Scopus, Science Direct and Online Journals, and includes 127 references.

Results: A survey of literature revealed that extracts/fractions/isolates from 18 plants, 23 chemical constituents of plant origin and 40 plant-based formulations from various systems of medicine have been reported to possess antipsoriatic activity, and 37 antipsoriatic formulations containing plants have been patented.

Conclusion: Preliminary antipsoriatic activity studies have been carried out on crude extracts of traditionally used and medicinally promising plants. Such plants need to be explored properly with a view to isolate antipsoriatic constituents, and to evaluate their possible mode of actions so that these plant drugs could be exploited properly as potential antipsoriatic drugs.

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Review > Arch Dermatol Res. 2014 Apr;306(3):211-29. doi: 10.1007/s00403-013-1428-4. Epub 2013 Nov 20.

Phytotherapy in the management of psoriasis: a review of the efficacy and safety of oral interventions and the pharmacological actions of the main plants

Shiqiang Deng [1], Brian H May, Anthony L Zhang, Chuanjian Lu, Charlie C L Xue

Affiliations + expand

PMID: 24253308 DOI: 10.1007/s00403-013-1428-4

COMUNIDADE Saúde Conectada

Abstract

This review provides assessments of the efficacy and safety of oral forms of phytotherapy in psoriasis management and discusses the pharmacological actions of the plants that have been frequently used in clinical trials. It employed the methods described in the Cochrane Handbook. Ten randomized controlled trials that compared a plant-based intervention with placebo or a pharmacotherapy in the treatment of psoriasis vulgaris and used Psoriasis Area Severity Index (PASI) as an outcome measure were included. Superiority to placebo was found in two out of three studies. In six out of seven studies, the effect of the phytotherapy was reported as comparable to the pharmacotherapy in the short term when assessed as PASI 50. The safety of the phytotherapies was discussed. The most commonly used plants were Oldenlandia diffusa, Rehmannia glutinosa and Salvia miltiorrhiza. Experimental studies on extracts and compounds derived from these plants have reported anti-inflammatory, anti-proliferative and other actions of relevance to psoriasis management. These properties may account for the apparent actions of some of the phytotherapies used in these clinical studies. These plants and their active constituents appear to warrant further research attention in the search for future medications for psoriasis.

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CSC

Como usar ?

1. Escolher DOIS suplementos para PSORÍASE
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar
5. Pode tomar com chá de matricária, feno grego, bardana ou verde

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Chá verde - ECGC, Ltheanina
Sabugueiro (banho)
Camomila

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> BMC Complement Altern Med. 2016 Aug 31;16(1):334. doi: 10.1186/s12906-016-1325-4.

Epigallocatechin-3-gallate (EGCG) inhibits imiquimod-induced psoriasis-like inflammation of BALB/c mice

Shuangshuang Zhang [1], Xiangdong Liu [2], Lihong Mei [1], Hongfeng Wang [1], Fang Fang [3]

Affiliations + expand

PMID: 27581210 PMCID: PMC5007807 DOI: 10.1186/s12906-016-1325-4

> Front Pharmacol. 2021 Jul 26;12:719842. doi: 10.3389/fphar.2021.719842. eCollection 2021.

L-Theanine Alleviates IMQ-Induced Psoriasis Like Skin Inflammation by Downregulating the Production of IL-23 and Chemokines

Yaohan Xu [1], Jiang Zhu [1], Jingyi Hu [2], Ziqi Zou [2], Yueling Zhao [3], Lihua Lai [2], Ping Xu [3], Yinjing Song [1], Hao Cheng [1]

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Gerânio
Lavanda
Melaleuca
Benjoim
Camomila

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> J Ethnopharmacol. 2020 Oct 28;261:113127. doi: 10.1016/j.jep.2020.113127. Epub 2020 Jul 3.

Anti-psoriatic effect of Lavandula angustifolia essential oil and its major components linalool and linalyl acetate

Vineet Kumar Rai [1], Priyam Sinha [2], Kuldeep Singh Yadav [3], Aparna Shukla [4], Archana Saxena [5], Dnyaneshwar Umrao Bawankule [6], Sudeep Tandon [7], Feroz Khan [8], Chandan Singh Chanotiya [9], Narayan Prasad Yadav [10]

Case Reports > Nurs Stand. 1996 Dec 18;11(13-15):53-6. doi: 10.7748/ns.11.15.53.s68.

Using aromatherapy in the management of psoriasis

D Walsh [1]

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Review > Dermatol Ther (Heidelb). 2022 Oct;12(10):2273-2283.
doi: 10.1007/s13555-022-00802-1. Epub 2022 Sep 14.

Mindfulness and Meditation for Psoriasis: A Systematic Review

Erin Bartholomew [1], Mimi Chung [2], Samuel Yeroushalmi [3], Marwa Hakimi [4], Tina Bhutani [4], Wilson Liao [4]

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Review > Psoriasis (Auckl). 2022 Mar 25;12:25-33. doi: 10.2147/PTT.S326327.
eCollection 2022.

Assessing and Improving Psychological Well-Being in Psoriasis: Considerations for the Clinician

Brittany Blackstone [#1], Radhika Patel [#1], Anthony Bewley [2]

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Randomized Controlled Trial > Nutrients. 2022 Oct 25;14(21):4470. doi: 10.3390/nu14214470.

Efficacy and Safety of Fig (*Ficus carica* L.) Leaf Tea in Adults with Mild Atopic Dermatitis: A Double-Blind, Randomized, Placebo-Controlled Preliminary Trial

Tatsuya Abe [1], Yukari Koyama [1], Kosaku Nishimura [1], Aya Okiura [1], Toru Takahashi [1]

Affiliations + expand

PMID: 36364740 PMCID: PMC9658579 DOI: 10.3390/nu14214470

Free PMC article

Abstract

Atopic dermatitis (AD) is a chronic, recurrent pruritic skin disease with repeated remissions and exacerbations. Various factors, such as allergies, skin conditions and lifestyle, combine to cause AD, making it difficult to cure completely. Although AD symptoms are suppressed with medications, this is a long-term effort and burden on patients. Thus, safer drugs and alternatives are needed. We previously found that consumption of tea prepared from fig (*Ficus carica* L.) leaves alleviated allergy and AD symptoms in cultured cells and animals. Therefore, here, we conducted a double-blind, randomized, controlled study in patients with mild AD to evaluate the safety and AD-relieving effects of prolonged consumption of fig leaf tea. Positive effects of fig leaf tea consumption were confirmed in 14 of 15 participants. Eczema Area and Severity Index values were significantly lowered in the fig leaf tea-treated group than in the placebo-treated group. The effect weakened 4 weeks after the end of the intervention, suggesting that continued intake of fig leaf tea was effective. Further assessments confirmed the safety of fig leaf tea consumption and revealed no variations that might pose a health hazard. Therefore, we postulate that fig leaf tea is a natural and safe therapeutic option for AD.

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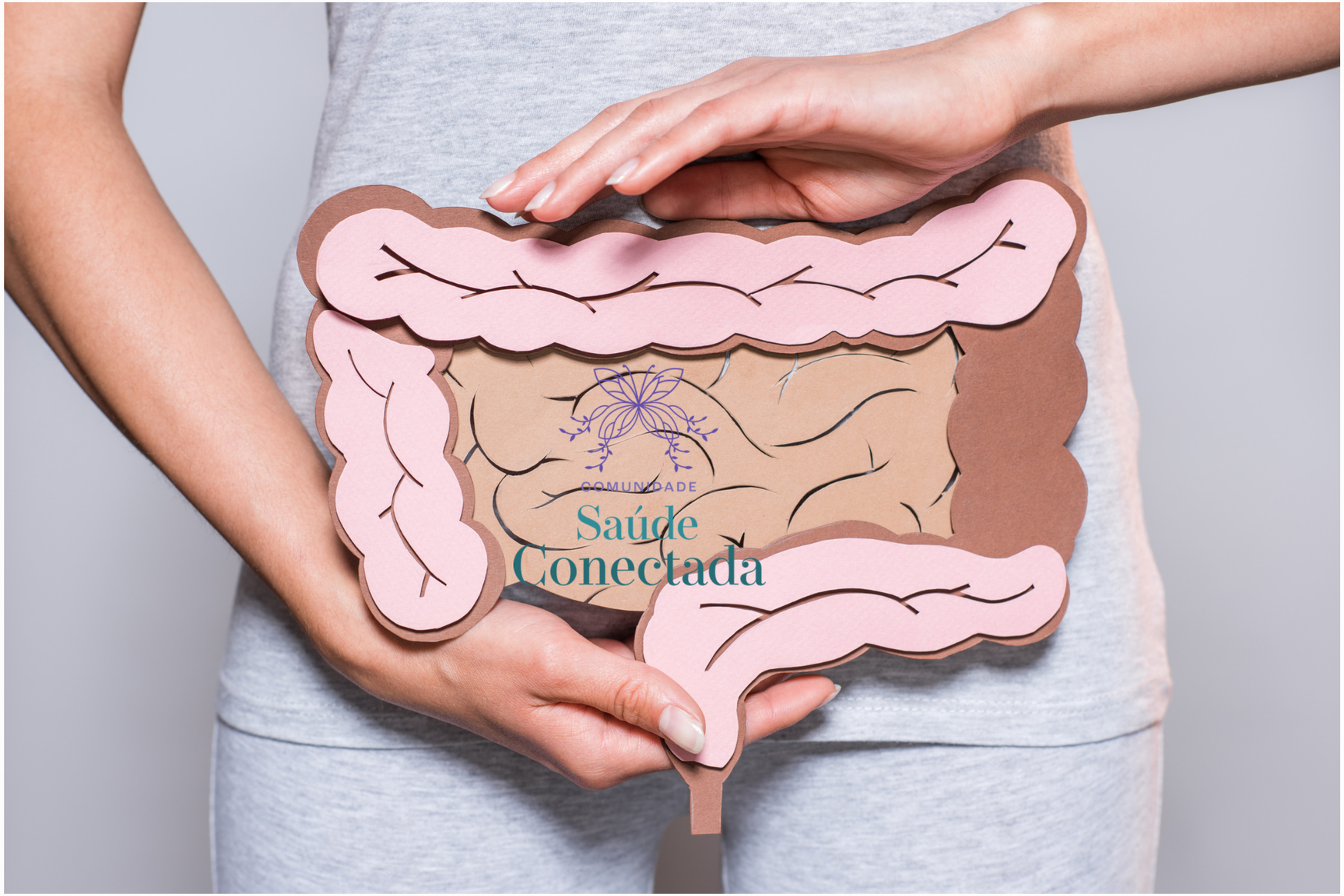

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Review > Int J Mol Sci. 2021 Apr 13;22(8):3998. doi: 10.3390/ijms22083998.

Alterations of the Skin and Gut Microbiome in Psoriasis and Psoriatic Arthritis

Irmina Olejniczak-Staruch [1 2], Magdalena Ciążyńska [3], Dorota Sobolewska-Sztychny [1], Joanna Narbutt [1], Małgorzata Skibińska [1], Aleksandra Lesiak [1]

Affiliations + expand

PMID: 33924414 PMCID: PMC8069836 DOI: 10.3390/ijms22083998

Free PMC article

Abstract

Numerous scientific studies in recent years have shown significant skin and gut dysbiosis among patients with psoriasis. A significant decrease in microbiome alpha-diversity (abundance of different bacterial taxa measured in one sample) as well as beta-diversity (microbial diversity in different samples) was noted in psoriasis skin. It has been proven that the representation of *Cutibacterium*, *Burkholderia* spp., and *Lactobacilli* is decreased and *Corynebacterium kroppenstedii*, *Corynebacterium simulans*, *Neisseria* spp., and *Finegoldia* spp. increased in the psoriasis skin in comparison to healthy skin. Alterations in the gut microbiome in psoriasis are similar to those observed in patients with inflammatory bowel disease. In those two diseases, the *F. prausnitzii*, *Bifidobacterium* spp., *Lactobacillus* spp., *Parabacteroides* and *Coprobacillus* were underrepresented, while the abundance of *Salmonella* sp., *Campylobacter* sp., *Helicobacter* sp., *Escherichia coli*, *Alcaligenes* sp., and *Mycobacterium* sp. was increased. Several research studies provided evidence for the significant influence of psoriasis treatments on the skin and gut microbiome and a positive influence of orally administered probiotics on the course of this dermatosis. Further research is needed to determine the influence of the microbiome on the development of inflammatory skin diseases. The changes in microbiome under psoriasis treatment can serve as a potential biomarker of positive response to the administered therapy.

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Randomized Controlled Trial > Acta Derm Venereol. 2019 Nov 1;99(12):1078-1084. doi: 10.2340/00015555-3305.

Efficacy and Safety of Oral Administration of a Mixture of Probiotic Strains in Patients with Psoriasis: A Randomized Controlled Clinical Trial

Vicente Navarro-López [1], Asunción Martínez-Andrés, Ana Ramírez-Boscá, Beatriz Ruzafa-Costas, Eva Núñez-Delegido, Miguel A Carrión-Gutiérrez, David Prieto-Merino, Francisco Codoñer-Cortés, Daniel Ramón-Vidal, Salvador Genovés-Martínez, Empar Chenoll-Cuadros, José M Pérez-Orquín, José A Picó-Monllor, Sara Chumillas-Lidón

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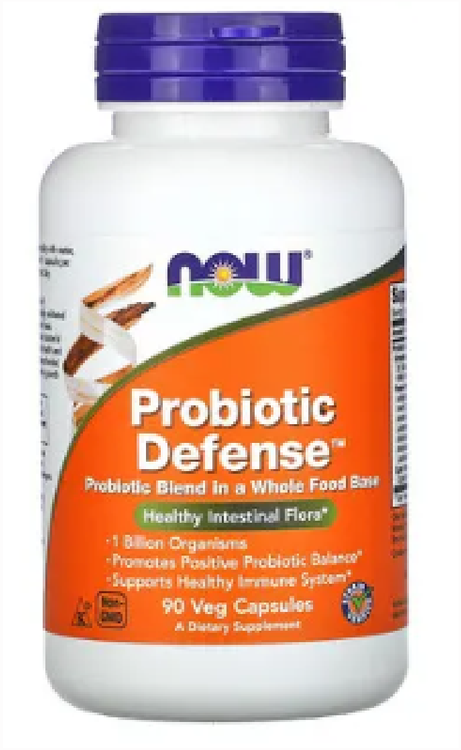

INGREDIENTES

Lactobacillus acidophilus NCFM (ATCC SD5221), Lactobacillus paracasei Lpc-37 (ATCC SD5275), Bifidobacterium animalis subsp lactis BI-04 (ATCC SD5219), Bifidobacterium animalis subsp lactis Bi-07 (ATCC SD5220), agente de massa celulose microcristalina, antiumectantes estearato de magnésio e dióxido de silício, cápsula (espessante hidroxipropilmetilcelulose).

NÃO CONTÉM GLÚTEN.

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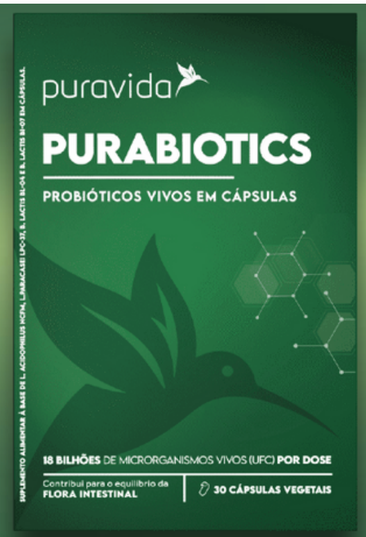
INFORMAÇÃO NUTRICIONAL

Porção de 1,7 g (3 cápsulas)

Quantidade por porção		%VD
Lactobacillus acidophilus NCFM	$4{,}5x10^9$ UFC	*
Lactobacillus paracasei Lpc-37	$4{,}5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis BI-04	$4{,}5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis Bi-07	$4{,}5x10^9$ UFC	*

Não contém quantidades significativas de valor energético, proteínas, carboidratos, gorduras totais, gorduras saturadas, gorduras trans, fibra alimentar e sódio. *VD não estabelecido.



SUGESTÃO DE USO

ADULTOS (MAIORES DE 19 ANOS), INGERIR 3 CÁPSULAS AO DIA.

USO NOTURNO, ANTES DE DORMIR

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CSC

Mascaras naturais

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Cúrcuma

Uma pasta de óleo de coco e açafrão pode ser aplicada na parte afetada.

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Azeite

Você aplica diretamente nas lesões de psoríase ou adiciona a um banho para aliviar o ressecamento e a irritação.

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Babosa.

Na medicina ayurvédica, o gel dessa planta é aplicado nas placas. Algumas pesquisas mostram que reduz a coceira e a descamação.

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Pasta de flor de jasmim para eliminar a coceira e reduzir a inflamação

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Manteiga de Karité
Este ingrediente não irritante e amigo da pele pode fornecer hidratação ideal para eczema de pele. Suas propriedades anti-inflamatórias acalmam ainda mais a pele irritada e ajudam a reparar a barreira cutânea.

Como usar manteiga de karité:

A manteiga de karité pode ser usada diariamente como um hidratante para o eczema da pele.

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Mix de calêndula e mel

Flores e folhas de calêndula são medicamentos antigos usados por suas propriedades cicatrizantes, anti-inflamatórias e antimicrobianas. Usá-lo em combinação com mel melhora ainda mais o tratamento do eczema.

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Como usar calêndula e mel:

Faça um mix moendo 4 a 5 flores e folhas de calêndula. Adicione o mel à mistura e aplique nas áreas afetadas. Lave após 15 a 20 minutos para curar o eczema leve.

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Café da manhã

Dia 1 - 2 panquecas de aveia com pasta de amêndoas e frutas picadas
Dia 2 - 2 unidades de torrada de arroz com 2 fatias de queijo branco + 1 laranja
Dia 3 - Farinha de aveia feita com leite de aveia ou coco e uma colher de sopa de sementes de chia + mistura de sementes

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Lanches

Opção 1 - ½ mamão + 1 colher de sopa de aveia
Opção 2 - 1 maçã cozida com canela, cravo e noz moscada
Opção 3 - 1 iogurte natural com 1 colher de sopa de sementes de linhaça e 6 nozes
Opção 4 - 1 smoothie feito com leite de aveia, morango e banana + 1 colher de sopa de sementes de chia
Opção 5 - Guacamole com cebola e pimentão + 2 fatias integrais de torrada
Opção 6 - 1 banana com canela

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Almoço e jantar

Dia 1 - 1 peito de frango grelhado com meia xícara de arroz integral e meia xícara de feijão, com uma salada de alface, pepino e tomate temperada com 1 colher de sopa de azeite + 1 fatia de abacaxi

Dia 2 - Macarrão de grão de bico com atum com salada de brócolis e cenoura temperada com 1 colher de sopa de azeite + 1 fatia de melão

Dia 3 - Peixe cozido com legumes + meia xícara de arroz integral + salada de legumes temperada com azeite extra virgem + 1 pêra

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Exemplo de suplementação

Onde comprar?

Site das marcas
Mercado Livre (ver pontuação do vendedor)
Amazon
Iherb

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